

GENERAL FIRST AID GUIDE

For Everyday Civilians — Standard Household First Aid Kit Assumed
Use Immediately While Awaiting Emergency Medical Services (EMS)

REFERENCES: American Heart Association (AHA) · American Red Cross · WHO First Aid Guidelines · Red Cross/Red Crescent International Guidelines

■ THIS GUIDE IS FOR EMERGENCY REFERENCE ONLY. Call EMS (911 or local emergency number) as soon as possible. This content is based on established first-aid protocols and does not replace trained medical personnel or professional medical advice.

CONDITIONS COVERED IN THIS GUIDE

1. Choking — Adult & Infant
2. Sudden Cardiac Arrest (CPR & AED)
3. Severe Allergic Reaction (Anaphylaxis)
4. Burns — Thermal & Chemical
5. Severe Bleeding & Deep Cuts
6. Suspected Fractures & Sprains
7. Seizures

How to Use This Guide

This guide is for people with **no medical training** who must act in an emergency before professional help arrives. Every section uses the same structure:

Section	What It Tells You
Overview	What the condition is and why it is dangerous
Signs & Symptoms	How to recognise the condition
Immediate Actions	What to do first — step by step, in order
Ongoing Care	What to keep doing until EMS arrives
What NOT to Do	Common mistakes that make things worse
When to Call EMS/Move	Explicit criteria for professional care



THE UNIVERSAL FIRST CHECK — DO THIS FOR EVERY EMERGENCY

Perform these 5 steps **before** treating any specific condition. They apply to every emergency.

1

ENSURE SCENE SAFETY

Do not approach until the environment is safe. Check for traffic, fire, unstable structures, electrical hazards, or ongoing threats. Your safety comes first — an injured responder helps no one.

2

CHECK RESPONSIVENESS

Shout the person's name and tap their shoulders firmly. If no response, they are unresponsive. Shout for help immediately.

3

CALL EMS NOW

Call 911 (or your local emergency number) immediately or direct a specific bystander: 'YOU — call 911 and come back to tell me.' Do not assume someone else has called.

4

CHECK BREATHING

Tilt the head back and look for chest rise for no more than 10 seconds. Normal breathing — proceed to step 5. No breathing — begin CPR (see Section 2).

5

CONTROL SEVERE BLEEDING

Look rapidly for blood soaking through clothing. Press hard with the cleanest material available. Then proceed to the specific condition section.

■ **ALWAYS ensure your own safety first. Never enter an unsafe scene to help someone.**

Assess before you act — a second casualty slows rescue and costs lives.

1 CHOKING — ADULT & INFANT

Choking occurs when a foreign object — food, a small toy, or fluid — partially or completely blocks the airway. A complete blockage stops all air exchange and causes unconsciousness within minutes. Immediate action prevents brain damage and death.

1A. Choking — Conscious Adult or Child (Over 1 Year)

Signs & Symptoms

- Sudden inability to speak, cry, or make sound
- Clutching the throat with one or both hands (universal choking sign)
- Weak, ineffective, or absent cough
- High-pitched wheezing or no breathing sounds at all
- Face turns red, then blue (cyanosis) — sign of severe hypoxia
- Panic, wide eyes, frantic gestures

Immediate Actions

1. **ASK:** 'Are you choking?' If the person can speak or cough forcefully — encourage them to keep coughing. Do not intervene yet.
2. **IF THEY CANNOT SPEAK OR COUGH:** Stand behind them. Lean them slightly forward.
3. **BACK BLOWS:** Give 5 firm blows between the shoulder blades using the heel of your hand. Each blow should be a separate, distinct strike.
4. **ABDOMINAL THRUSTS (HEIMLICH):** Place one fist above the navel and below the breastbone. Grip your fist with your other hand. Give 5 sharp inward-and-upward thrusts.
5. **ALTERNATE:** Repeat — 5 back blows, then 5 abdominal thrusts — until the object is expelled, the person can breathe and cough, or they lose consciousness.
6. **IF UNCONSCIOUS:** Lower them carefully to the ground. Call EMS if not already done. Begin CPR. Each time you open the airway to give a breath, look in the mouth — if you see an object, remove it.

Special Cases

- **Pregnant or very obese:** Use chest thrusts instead of abdominal thrusts — hands on the center of the chest, not the abdomen.
- **If alone and choking:** Call 911 first if possible. Attempt self-abdominal thrusts — or thrust your upper abdomen sharply against the back of a chair or hard edge.

1B. Choking — Infant (Under 1 Year)

Signs & Symptoms

- Sudden silence — no cry, no cough
- No visible chest rise
- Blue or dusky lips and skin

Immediate Actions

1. POSITION: Hold the infant face-down along your forearm, head lower than chest. Support the head firmly.
2. BACK BLOWS: Deliver 5 firm back blows between the shoulder blades using the heel of your hand.
3. FLIP: Turn the infant face-up on your other forearm, still head-lower-than-chest.
4. CHEST THRUSTS: Place 2 fingers on the center of the chest, just below the nipple line. Give 5 quick downward chest thrusts (NOT abdominal thrusts — never perform Heimlich on an infant).
5. CHECK MOUTH: Look in the mouth. Remove any visible object with your finger. Do not perform blind finger sweeps.
6. REPEAT: Alternate 5 back blows / 5 chest thrusts until the object is expelled or the infant loses consciousness.
7. IF UNCONSCIOUS: Begin infant CPR (see Section 2). Call EMS immediately.

Ongoing Care

If the object is expelled and the person is breathing: keep them calm and seated. Watch for continued coughing, difficulty swallowing, or voice changes — the object may still be partially lodged or airway tissue may be bruised.

■ WHAT NOT TO DO

Do NOT perform abdominal thrusts on infants under 1 year — this causes organ injury.

Do NOT perform blind finger sweeps — you may push the object deeper.

Do NOT give water or food after choking until the person is assessed by EMS.

Do NOT slap the back of a conscious infant while they are upright — always lean them forward and downward first.

When to Call EMS

- The object is not expelled after repeated cycles
- The person loses consciousness at any point
- Breathing does not return to normal after the object is cleared
- The victim is an infant — always call EMS after an infant choking episode

2 SUDDEN CARDIAC ARREST — CPR & AED

Sudden cardiac arrest (SCA) occurs when the heart stops beating effectively. Blood and oxygen no longer reach the brain. Brain damage begins within 4 minutes. Death follows within 10 minutes without intervention. CPR and early defibrillation (AED) are the only field interventions that save lives. Every minute without CPR reduces survival by 7–10%.

Signs & Symptoms

- Sudden collapse — person drops without warning
- Unresponsive — no reaction to shouts or shoulder tap
- No normal breathing — absent, gasping ('agonal breathing'), or gurgling
- No pulse detectable at the wrist or neck
- Lips, fingernails, or skin turning blue (cyanosis)
- May have a brief seizure-like shaking at onset

Immediate Actions — Adult CPR (Over 8 Years)

1. **CONFIRM UNRESPONSIVENESS:** Tap shoulders and shout. No response?
2. **CALL EMS:** Call 911. Shout for an AED. Direct a bystander by name: 'You — call 911. You — find the AED.'
3. **CHECK BREATHING:** Look for chest rise for no more than 10 seconds. Gasping is NOT normal breathing.
4. **POSITION:** Lay the person on their back on a hard, flat surface.
5. **HAND PLACEMENT:** Heel of one hand on the center of the chest (lower half of the breastbone). Second hand on top, fingers interlaced.
6. **COMPRESS:** Push straight down at least 5–6 cm (2–2.4 inches). Allow full chest recoil between compressions. Do not lean on the chest.
7. **RATE:** 100–120 compressions per minute. Count aloud: '1 and 2 and 3...' Think of a fast song beat.
8. **RESCUE BREATHS** (if trained and willing): After 30 compressions — tilt head back, lift chin, pinch nose, give 1 breath over 1 second just until chest rises. Give 2 breaths, then resume compressions. Ratio: 30:2.
9. **COMPRESSIONS-ONLY CPR:** If untrained or unwilling to give breaths — give continuous chest compressions at 100–120/min without stopping. This is effective and recommended.
10. **CONTINUE:** Until the AED arrives, EMS takes over, the person starts breathing normally, or you are physically unable to continue.

AED — Automated External Defibrillator

AEDs are found in airports, malls, gyms, offices, and schools. They provide voice instructions. Trust them.

1. **POWER ON:** Open the AED and press the power button. Follow voice prompts.
2. **ATTACH PADS:** Place pads exactly as shown on the diagram — one below the right collarbone, one on the left side of the chest below the armpit. Bare skin only. Cut or tear clothing away.
3. **CLEAR AND ANALYZE:** Stop CPR. Ensure no one is touching the person. The AED analyzes the heart rhythm.
4. **DELIVER SHOCK:** If a shock is advised — shout 'CLEAR,' visually confirm no one is touching the person, then press the shock button.

5. RESUME CPR: Immediately after the shock (or if no shock advised), resume CPR for 2 minutes before the AED re-analyzes.
6. REPEAT: Continue CPR + AED cycles until EMS arrives or the person recovers.

CPR — Infant (Under 1 Year)

1. 2 FINGERS: Place the middle and ring finger on the center of the chest, just below the nipple line.
2. COMPRESS: Push down approximately 4 cm (1.5 inches). Allow full recoil.
3. RATE: 100–120 compressions per minute.
4. BREATHS: Cover both the baby's mouth AND nose with your mouth. Give 2 gentle puffs — just enough to see the chest rise. Ratio: 30:2.

CPR — Child (1–8 Years)

- Use the heel of ONE hand. Compress 5 cm (2 inches). Rate 100–120/min. 30:2 ratio.

Ongoing Care

Do not stop CPR until EMS arrives or the person shows clear signs of life (normal breathing, movement, coughing). If the person revives — keep them still, warm, and calm. Do not give food or water. Maintain monitoring until EMS arrives.

■ WHAT NOT TO DO

Do NOT begin CPR on a person who is breathing normally — it can cause cardiac problems.

Do NOT stop CPR to check for pulse more than once every 2 minutes.

Do NOT remove AED pads once applied.

Do NOT wait for the AED before starting CPR — start immediately and use the AED as soon as available.

When to Call EMS

Call EMS immediately for any cardiac arrest. EMS must be en route before or during CPR.

3 SEVERE ALLERGIC REACTION — ANAPHYLAXIS

Anaphylaxis is a severe, whole-body allergic reaction that progresses rapidly to airway closure, circulatory collapse, and death — often within minutes. Common triggers include foods (peanuts, shellfish, tree nuts), insect stings, latex, and medications. Epinephrine (adrenaline) is the only effective treatment. Antihistamines alone are insufficient and dangerously slow.

Signs & Symptoms

Symptoms appear within seconds to 30 minutes of exposure. Look for two or more body systems involved:

- **Skin:** Hives, flushing, swelling of face, lips, tongue, or throat
- **Airway:** Hoarse voice, stridor (high-pitched breathing), difficulty swallowing
- **Breathing:** Wheezing, shortness of breath, feeling of chest tightness
- **Circulation:** Rapid weak pulse, pale or blue skin, sudden drop in blood pressure, dizziness
- **Gut:** Nausea, vomiting, severe cramping — especially combined with skin or breathing symptoms
- **Neurological:** Sudden anxiety, confusion, loss of consciousness

Immediate Actions

1. **CALL EMS IMMEDIATELY:** Anaphylaxis is always a 911 emergency — even if symptoms seem mild at first.
2. **IDENTIFY EPINEPHRINE AUTO-INJECTOR:** Ask if the person carries an EpiPen, Auvi-Q, or similar device. Check their bag, belt clip, or pockets.
3. **ADMINISTER EPINEPHRINE:** Hold the auto-injector in your dominant hand. Remove the safety cap. Press the tip firmly against the outer mid-thigh (through clothing is fine). Hold for 3 seconds. The device clicks. Remove. Note the time.
4. **POSITION:** If breathing is difficult — sit them upright. If pale, dizzy, or faint — lay them flat with legs elevated. If unconscious and breathing — recovery position on side.
5. **DO NOT LET THEM STAND OR WALK:** Standing causes blood to pool in the legs and can cause sudden cardiovascular collapse ('empty ventricle syndrome').
6. **SECOND DOSE:** If symptoms do not improve or worsen after 5–15 minutes and a second auto-injector is available, administer it in the opposite thigh.
7. **BE READY FOR CPR:** If breathing stops or no pulse is felt — begin CPR immediately (see Section 2).

Ongoing Care

Stay with the person continuously. Anaphylaxis can have a biphasic reaction — symptoms may return 1–12 hours after apparent recovery. All anaphylaxis cases must be transported to hospital by EMS, even after epinephrine improves symptoms.

■ WHAT NOT TO DO

- Do NOT give antihistamines (Benadryl) as a substitute for epinephrine — they act too slowly for anaphylaxis.
- Do NOT allow the person to stand or walk if they appear pale or faint.
- Do NOT assume the person has recovered because symptoms briefly improve.
- Do NOT delay epinephrine — fear of giving an injection is far less dangerous than delaying it.

4 BURNS — THERMAL & CHEMICAL

Burns damage the skin and underlying tissue through heat, chemicals, electricity, or radiation. Severity is classified by depth. Cooling is the only first-aid measure proven to change burn outcomes. Chemical burns require continuous flushing. Incorrect home remedies (butter, toothpaste, ice) dramatically increase infection risk and worsen tissue damage.

Burn Classification

Degree	Depth	Appearance	Pain
1st Degree (Minor)	Surface only	Red, dry, no blisters — like a sunburn	Painful
2nd Degree (Moderate)	Into dermis	Blisters, wet-looking, deep red or pale	Very painful
3rd Degree (Severe)	Full thickness	White, brown, or black; leathery; dry	May be painless

4A. Thermal Burns — Immediate Actions

1. **STOP THE BURNING:** Remove the person from the heat source. Remove burning or smoldering clothing — unless it is stuck to the skin, in which case cut around it, never pull.
2. **COOL THE BURN:** Run cool (not cold, not icy) water over the burn for a full 10–20 minutes. Start within 3 hours of injury. This is the single most important intervention.
3. **REMOVE:** Gently remove rings, watches, or tight clothing near the burn before swelling begins. Do not remove if stuck to skin.
4. **COVER LOOSELY:** After cooling, cover with a clean non-fluffy material — cling film (plastic wrap) is ideal. A clean plastic bag works for hands. Do not wrap tightly.
5. **DO NOT BURST BLISTERS:** Blisters are a natural protective layer. Intact blisters reduce infection risk significantly.
6. **TREAT FOR SHOCK:** Large or deep burns cause massive fluid loss. Lay flat, keep warm if shivering, monitor for shock signs.

4B. Chemical Burns — Immediate Actions

1. **PROTECT YOURSELF:** Put on gloves or use a plastic bag before handling the patient or their clothing. Chemical burns can burn through your skin too.
2. **REMOVE CLOTHING:** Remove all contaminated clothing, shoes, and jewellery carefully. Do not allow chemical contact with unaffected skin.
3. **FLUSH CONTINUOUSLY:** Rinse the affected area with large amounts of cool running water for at least 20 minutes for acids, and 60 minutes or more for alkalis (bleach, lime, cement). Do not stop early.
4. **DO NOT NEUTRALISE:** Do not apply baking soda to acid burns or vinegar to alkali burns. Neutralisation reactions generate heat and worsen injury.
5. **COVER:** After flushing, cover loosely with clean dry material.
6. **CALL POISON CONTROL:** In addition to EMS, call poison control if the chemical is known — they can advise on specific antidotes or extended flushing times.

4C. Face or Inhalation Burns — Critical Signs

■ AIRWAY BURN — CRITICAL EMERGENCY

Singed eyebrows or nasal hair, hoarse voice, difficulty swallowing, or soot around the mouth/nose indicate a possible internal airway burn. The airway can swell shut within minutes to hours.

Sit the person upright. Call EMS immediately. Do NOT wait for breathing difficulty to worsen.

Ongoing Care

Keep the burned area elevated if possible. Keep the person warm — burned skin loses heat rapidly. Offer small sips of water if conscious and no airway injury is suspected. Do not remove the covering or re-expose the burn.

■ WHAT NOT TO DO

Do NOT apply butter, oil, toothpaste, egg white, or any home remedy — all cause infection and seal in heat.

Do NOT use ice or ice-cold water — causes vasoconstriction and extends tissue damage.

Do NOT burst blisters.

Do NOT remove clothing that is stuck to burned skin — cut around it.

Do NOT use fluffy cotton wool or adhesive bandages directly on burns.

When to Call EMS

- Burns larger than the palm of the person's hand (approximately 1% body surface area)
- Burns on the face, hands, feet, genitals, or over a major joint
- Any 3rd degree burn, or suspected deep burn
- Any chemical burn to the eyes
- Suspected inhalation burn or airway involvement
- Burns in children under 5 or adults over 60
- Burn caused by electricity — always requires hospital evaluation even if minor-looking

5 SEVERE BLEEDING & DEEP CUTS

Severe, uncontrolled bleeding is one of the most preventable causes of death. An adult can lose a fatal amount of blood in as little as 2–3 minutes from a major vessel injury. Direct, firm, uninterrupted pressure is the primary life-saving action. Improvised tourniquets on limbs are effective and appropriate when direct pressure fails.

Signs & Symptoms

- Blood soaking rapidly through clothing — a soaked cloth in under 1 minute is severe
- Blood spurting with each heartbeat (arterial bleeding — the most dangerous type)
- Signs of shock: pale skin, cold clammy sweat, rapid weak pulse, confusion, thirst
- Large wound with visible deep tissue, muscle, or bone

Immediate Actions

1. **PROTECT YOURSELF:** Put on disposable gloves from your first aid kit if immediately available. If not — act anyway. Do not delay for gloves.
2. **EXPOSE THE WOUND:** Cut or remove clothing around the wound to see it clearly and control it effectively.
3. **APPLY DIRECT PRESSURE:** Place the cleanest available material (sterile gauze pad, clean cloth) directly over the wound. Press as hard as you can with your palm or both hands.
4. **PACK DEEP WOUNDS:** If the wound is a deep hole or cavity — push clean gauze or cloth firmly into the wound with your fingers. Then press hard on top. This is called wound packing.
5. **HOLD PRESSURE:** Maintain continuous firm pressure for a minimum of 10 full minutes without lifting to check. If blood soaks through — add more material on top. Do NOT remove the first layer.
6. **TOURNIQUET FOR LIMB WOUNDS:** If bleeding from an arm or leg cannot be controlled with pressure after 2–3 minutes — apply a tourniquet (see below).
7. **ELEVATE:** If the wound is on a limb and no fracture is suspected — raise it above the level of the heart to slow blood flow.
8. **KEEP THEM CALM AND WARM:** Fear increases heart rate and blood loss. Anxiety worsens shock.

Improvised Tourniquet — Limbs Only

Use when direct pressure fails on an arm or leg wound. A tourniquet saves lives. Apply without hesitation when needed.

1. **MATERIAL:** Use a strip of cloth at least 4 cm (2 inches) wide — a torn shirt sleeve, belt, or scarf. Never use wire, rope, or narrow cord — they cut tissue.
2. **PLACEMENT:** Wrap 5–8 cm (2–3 inches) ABOVE the wound (toward the body, not below).
3. **TIGHTEN:** Tie a half-knot, place a stick or pen on the knot, tie a second knot over it. Twist the stick until bleeding stops. The person will feel severe pain — this is expected and normal.
4. **SECURE:** Tie the stick in place so it cannot unwind.
5. **RECORD TIME:** Write the time of application on the person's forehead or arm with a pen, marker, or anything available. Tell EMS when it was applied.
6. **DO NOT REMOVE:** Only trained medical personnel should remove a tourniquet.

Embedded Objects

- If an object (knife, glass, debris) is embedded in the wound — DO NOT remove it. It may be stemming blood flow. Pad around it to stabilise it in place without pressing on it.

Ongoing Care

Monitor pulse every few minutes. Signs of worsening shock (pale, cold, confused) require EMS urgently. Keep the person lying flat with legs elevated unless injury prevents it. Keep them warm and calm.

■ WHAT NOT TO DO

Do NOT remove a blood-soaked dressing — this disrupts the clot forming underneath. Add on top.

Do NOT use a tourniquet on the neck, chest, abdomen, or groin — limbs only.

Do NOT remove an embedded object.

Do NOT apply a tourniquet using wire, rope, or thin cord — they cause tissue damage without stopping bleeding.

When to Call EMS

- Bleeding cannot be controlled with 10 minutes of direct pressure
- Spurting arterial bleeding
- Deep wounds with visible tissue, tendon, muscle, or bone
- Any signs of shock develop
- Wound is on the head, neck, chest, abdomen, or groin

6 SUSPECTED FRACTURES & SPRAINS

A fracture is a break in a bone. A sprain is a tear or stretch of ligaments around a joint. Without imaging, it is often impossible to distinguish between them in the field. Treat any suspected fracture as confirmed. Open fractures (bone visible) carry serious infection risk and require urgent medical attention. Mishandling a fracture — especially of the spine — can cause permanent paralysis.

Signs & Symptoms

- **Both fracture and sprain:** Pain, swelling, bruising at the injury site
- **Fracture-specific:** Deformity — limb looks bent, shortened, or out of position
- Grinding or grating sensation when moved (do not test deliberately)
- Complete inability or strong refusal to use or bear weight on the limb
- Bone visibly protruding through the skin (open/compound fracture)
- Numbness or tingling below the injury (nerve involvement)

Immediate Actions

1. **STOP ALL MOVEMENT:** Tell the person to stay still. Do not try to straighten the limb.
2. **CONTROL BLEEDING:** If there is an open wound — apply gentle direct pressure around (not on) the wound. Do not push exposed bone back in.
3. **SPLINT IN PLACE:** Immobilise the limb in the position you find it. Use any rigid material alongside it — a folded newspaper, rolled clothing, or sticks. Tie above AND below the fracture site — not over it.
4. **PAD THE SPLINT:** Add soft padding between the rigid splint and the limb for comfort.
5. **CHECK CIRCULATION:** After splinting — feel that fingers or toes below the fracture are warm. Can the person feel you touching them? If cold, pale, or numb — the splint is too tight. Loosen slightly.
6. **ELEVATE:** If possible, rest the injured limb elevated above heart level to reduce swelling.
7. **COVER OPEN FRACTURES:** If bone is visible, cover loosely with a clean, damp cloth. Do not attempt to push bone back. This is a high infection risk.
8. **ICE (if available):** Apply a cold pack or ice wrapped in cloth for 20 minutes to reduce swelling. Never apply ice directly to skin.

Suspected Spinal Injury — CRITICAL CAUTION

■ DO NOT MOVE A PERSON WITH A SUSPECTED SPINAL INJURY.

Suspect spinal injury after: high-speed accident, fall from height, diving injury, or any trauma with neck pain, back pain, or tingling/numbness in limbs.

Keep the head and neck perfectly still. Do not allow the person to sit up.

Only move if the scene becomes life-threatening and you have no other option.

If you must move them — keep the head, neck, and spine aligned in a straight line at all times.

Ongoing Care

Keep the person still and warm. Re-check circulation distal to the splint every 10 minutes. Do not give food or water if surgery may be needed. Reassure and monitor for signs of shock.

■ WHAT NOT TO DO

Do NOT attempt to realign, straighten, or 'reset' a fractured limb.

Do NOT apply ice directly to skin — wrap it first.

Do NOT move a person with suspected spinal injury unless their life is in immediate danger.

Do NOT ignore circulation checks after splinting — impaired circulation can cause permanent damage.

When to Call EMS

- Any suspected fracture of the spine, pelvis, or femur (thigh bone) — these are EMS emergencies
- Open fracture — bone visible
- Circulation below the fracture is impaired (cold, pale, numb) after splinting
- Signs of shock are present
- The person cannot bear weight at all after a lower limb injury

7 SEIZURES

A seizure is a sudden, uncontrolled electrical disturbance in the brain that causes changes in behaviour, movement, consciousness, or sensation. Most seizures in people with epilepsy end on their own within 1–3 minutes. The primary role of a bystander is to protect the person from injury and monitor the situation. Attempting to restrain a seizure or put anything in the mouth causes more harm than the seizure itself.

Signs & Symptoms

- **Convulsive (Tonic-Clonic/Grand Mal):** Sudden loss of consciousness, body stiffens (tonic phase), followed by rhythmic jerking of limbs (clonic phase)
- **Before the seizure:** Some people have a 'warning' (aura) — strange smell, vision changes, déjà vu, or sudden fear
- Biting of the tongue or inside cheek (do NOT put anything in mouth to prevent this)
- Loss of bladder or bowel control
- Confusion, disorientation, or drowsiness after the seizure ends (post-ictal state) — normal, lasts minutes to an hour
- **Absence/Focal seizures:** Brief staring spells, subtle repetitive movements (lip-smacking, hand-rubbing), altered awareness

Immediate Actions

1. **TIME THE SEIZURE:** Note when it starts. This is critical information for EMS. A seizure lasting more than 5 minutes is a medical emergency (status epilepticus).
2. **CLEAR THE AREA:** Move hard, sharp, or hot objects away from the person. If they are near stairs or a road — protect them, but do not force them to move.
3. **CUSHION THE HEAD:** Place something soft under their head (folded jacket, bag). Turn them gently onto their side if possible — this keeps the airway clear and allows fluids to drain.
4. **DO NOT RESTRAIN:** Never hold the person down or try to stop the movements. Restraint causes injury. The movements cannot be stopped.
5. **DO NOT PUT ANYTHING IN THE MOUTH:** The tongue cannot be swallowed. Putting fingers or objects in the mouth causes broken teeth, broken fingers, and airway injury.
6. **STAY WITH THEM:** Monitor breathing throughout. Do not leave.
7. **AFTER THE SEIZURE:** Turn them onto their side (recovery position). Talk calmly. They will be confused and frightened. Tell them what happened.
8. **CHECK BREATHING:** Most people breathe normally after a seizure. If not — open the airway and begin rescue breaths (see Section 2 / Section 5A).

Ongoing Care

Remain with the person until they are fully conscious and aware. The post-ictal period (confusion after a seizure) is normal and can last up to 30–60 minutes. Do not offer food, water, or medication until fully alert. If they have a medical bracelet or emergency information card — read it.

■ WHAT NOT TO DO

Do NOT restrain the person or try to stop the movements.

Do NOT put fingers, a spoon, a wallet, or any object in their mouth.

Do NOT give water, food, or medication during or immediately after the seizure.

Do NOT leave them unattended during or in the immediate aftermath.

Do NOT assume recovery — monitor breathing closely in the post-ictal phase.

When to Call EMS

- The seizure lasts more than 5 minutes — call immediately
- A second seizure follows without full recovery between them
- The person does not regain consciousness within 10 minutes
- Breathing does not return after the seizure ends
- The person is injured during the seizure
- First-ever seizure — even if it resolves
- The person is pregnant
- The seizure occurred in water



QUICK REFERENCE SUMMARY

Scan this table in seconds. Find the condition, read the first action, then go to the full section.

Condition	First Action
Choking — adult, conscious	Ask 'Are you choking?' → 5 back blows → 5 abdominal thrusts. Alternate until cleared.
Choking — infant	5 back blows (face-down) → 5 chest thrusts (face-up). Never abdominal thrusts on infant.
Cardiac arrest — adult	Call EMS + request AED → 30 chest compressions (5–6 cm) at 100–120/min → 2 breaths.
Cardiac arrest — infant	2-finger compressions, 4 cm deep, 100–120/min → 2 gentle puffs over mouth AND nose.
AED available	Power on → attach pads as shown → clear and analyze → shock if advised → resume CPR.
Anaphylaxis	Call EMS immediately → administer epinephrine auto-injector into outer thigh → sit or lie flat.
Thermal burn	Cool under running water for 10–20 minutes → cover loosely. No butter, oil, or ice.
Chemical burn	Remove contaminated clothing → flush with running water for 20–60+ minutes. Call Poison Control.
Airway burn (face/inhalation)	Sit upright → call EMS immediately. Airway can swell shut rapidly.
Severe bleeding — limb	Direct firm pressure for 10 minutes. If uncontrolled → apply tourniquet 5 cm above wound.
Severe bleeding — embedded object	Pad around the object to stabilize it. Do NOT remove it.
Suspected fracture	Immobilise in the position found. Splint above and below fracture. Check circulation.
Open fracture (bone visible)	Cover loosely with clean damp cloth. Do not push bone back. Call EMS.
Suspected spinal injury	Do NOT move. Keep head and neck still. Call EMS. Only move if scene is life-threatening.
Seizure	Clear area, cushion head, time the seizure. Do NOT restrain. Do NOT put anything in mouth.
Seizure > 5 minutes	Call EMS immediately — this is status epilepticus.
Post-seizure	Recovery position. Monitor breathing. Stay calm. Expect 10–30 min confusion.
Any emergency — all	Scene safe? → Check responsiveness → Call EMS → Check breathing → Control bleeding.

REFERENCES & SOURCES

All protocols in this guide are derived from the following authoritative sources. Consult primary sources for full clinical detail.

- **American Heart Association (AHA)**

2020 AHA Guidelines for CPR and Emergency Cardiovascular Care. Circulation, 2020. Includes adult, child, and infant CPR; AED use; choking management.

- **American Red Cross**

First Aid/CPR/AED Participant's Manual. American Red Cross, 2021. Includes choking, bleeding control, burns, fractures, and seizure management.

- **World Health Organization (WHO)**

Basic Emergency Care: Approach to the Acutely Ill and Injured. Geneva: WHO, 2018. Core triage and field stabilization principles.

- **International Federation of Red Cross and Red Crescent Societies**

International First Aid, Resuscitation, and Education Guidelines. IFRC, 2020. Globally applicable first aid protocols including burns, fractures, seizures, and allergic reactions.

- **American College of Surgeons**

Advanced Trauma Life Support (ATLS), 10th Edition. Chicago: ACS, 2018. Bleeding control, shock, airway management, and fracture principles.

- **Anaphylaxis Campaign / NICE Guidelines**

NICE Guideline NG80: Anaphylaxis. National Institute for Health and Care Excellence, UK, 2021. Epinephrine administration, anaphylaxis recognition, and biphasic reactions.

- **British Burn Association / WHO**

WHO Burns Management Guidelines; European Burns Association First Aid Guidance, 2019. Thermal and chemical burn cooling, dressing, and airway burn criteria.

- **Epilepsy Foundation**

Seizure First Aid Guidelines. Epilepsy Foundation of America, 2019. Seizure types, duration thresholds for EMS, bystander safety.

- **Committee on Tactical Combat Casualty Care (CoTCCC)**

TCCC Guidelines for Medical Personnel, 2023. Joint Trauma System, US Department of Defense. Tourniquet application, wound packing, and hemorrhage control.

- **Prehospital Trauma Life Support (PHTLS)**

PHTLS: Prehospital Trauma Life Support, 9th Edition. Jones & Bartlett, 2019. Fracture splinting, spinal precautions, bleeding control, and shock.